

Neuro-Bridging

A TRANSFORMATIVE NEW CATEGORY OF DIGITAL
THERAPEUTIC

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Neuro-Bridging

Neuro-bridging eliminates PTSD symptoms through an
imagery-based technique grounded in memory
reconsolidation

THE PROBLEM

£117.9 billion

**COST OF MENTAL
HEALTH
PROBLEMS TO UK
ECONOMY**

\$1 Trillion

**Total World
Cost of Mental
health**

\$232.2 Billion

**Cost of PTSD
in the USA**

**DEMAND FOR THERAPY WILL ALWAYS OUTSTRIP THE SUPPLY OF
THERAPISTS**

THE PROBLEM-DETAIL

- Most forms of psychotherapy do not create transformational change for clients
- Long waiting times to access therapy
- Professionals risk burnout and vicarious trauma
- Training therapists is costly and resource-intensive
- Urgent need for more accessible & efficient treatments

The Answer: Memory Reconsolidation

What is Memory Reconsolidation?

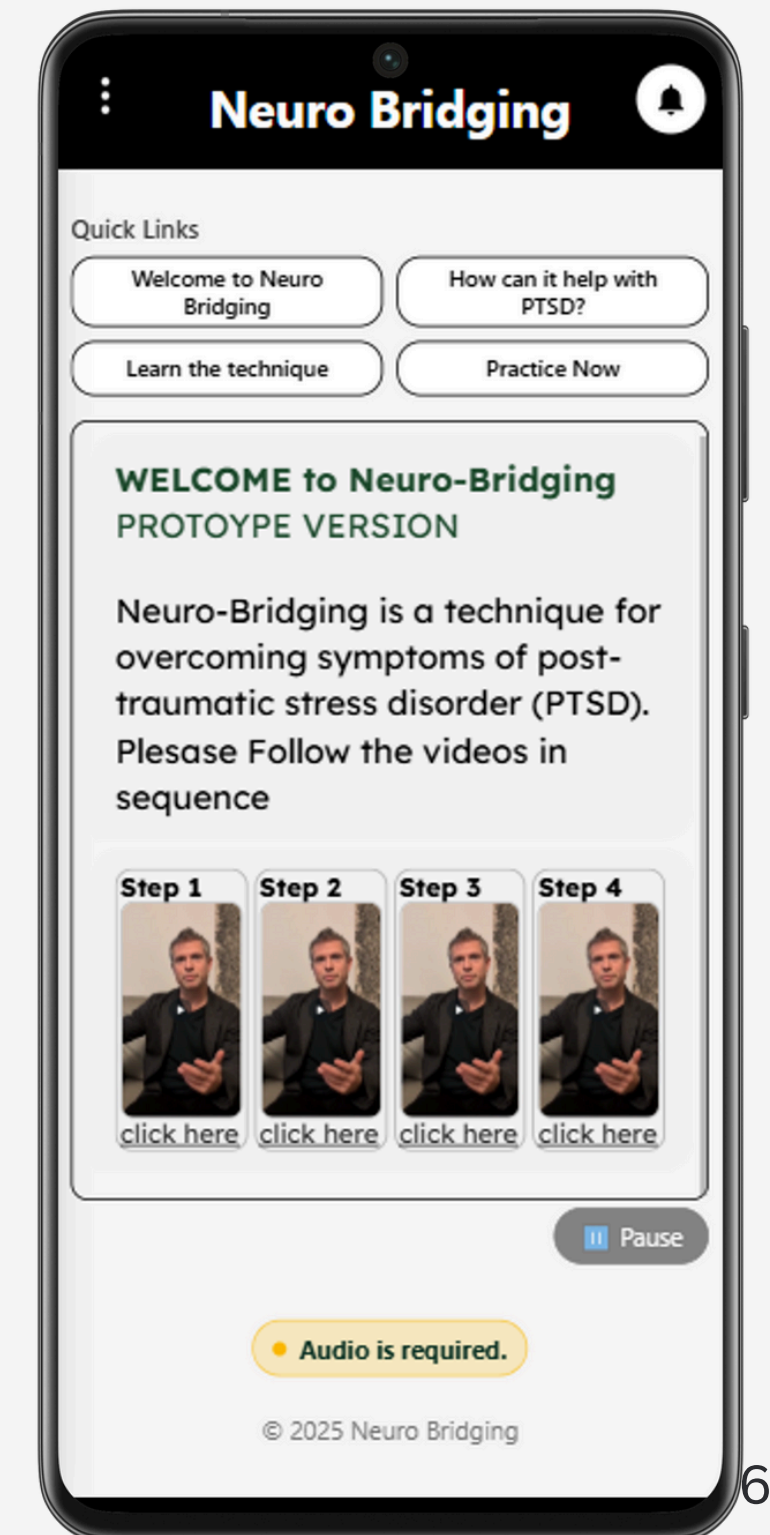
- A neurobiological process in which long-term memories become labile when recalled, allowing them to be modified
- First empirically demonstrated in 2000 (Nader, Schafe & LeDoux, Nature)
- Documented in Nature Reviews Neuroscience (Nader & Hardt, 2009)
- Enables rewiring of neural circuits
- Reactivating an old, stable memory makes it briefly plastic and changeable
- Facilitates editing of emotional memory
- Emotional memory drives many mental health conditions – e.g. triggering flashbacks, nightmares, and hyperarousal in PTSD

THE SOLUTION - A SUCCESSFUL THERAPEUTIC TECHNIQUE DELIVERED DIGITALLY

Our prototype launches Neuro-Bridging as a digital scalable product, allowing for widespread distribution of a structured treatment protocol developed and validated in real-world therapeutic settings

Delivers the technique to users through a structured workflow that integrates:

- Natural Language Processing-based training system
- Built-in scheduling and reminders
- Minimal data usage and on-device privacy protection
- Can be tuned incrementally
- Digital voice generation using 11 Labs
- Hosted on Google cloud
- Google gemini for nlp



NEURO-BRIDGING CLINICAL MEASUREMENT

Clear measurement happens at key stages

- Pre-measure: A clinical rating is recorded immediately before the first Neuro-Bridging practice.
- Post-measure: The same clinical rating is recorded five days after the practice period.
- Outcome comparison: Pre- and post-scores are compared to assess change.

The image shows two smartphone screens side-by-side, both displaying the 'Neuro Bridging' app interface. The left screen shows the 'PCL-5 - PTSD Checklist for DSM-5' with instructions and a list of symptoms. The right screen shows the 'PCL-5 total symptom score' of 60 / 80, with a 'Continue' button and a disclaimer.

Neuro Bridging

PCL-5 - PTSD Checklist for DSM-5

Below is a list of problems that people sometimes have in response to a very stressful experience. Keeping your worst event in mind, please read each problem carefully and select how much you have been bothered by that problem in the past month.

In the past month, how much were you bothered by:

1. Repeated, disturbing, and unwanted memories of the stressful experience?

☐ 0 - Not at all

☐ 1 - A little bit

☐ 2 - Moderately

☐ 3 - Quite a bit

☐ 4 - Extremely

2. Repeated, disturbing dreams of the stressful experience?

☐ 0 - Not at all

☐ 1 - A little bit

Neuro Bridging

60
/ 80

PCL-5 total symptom score

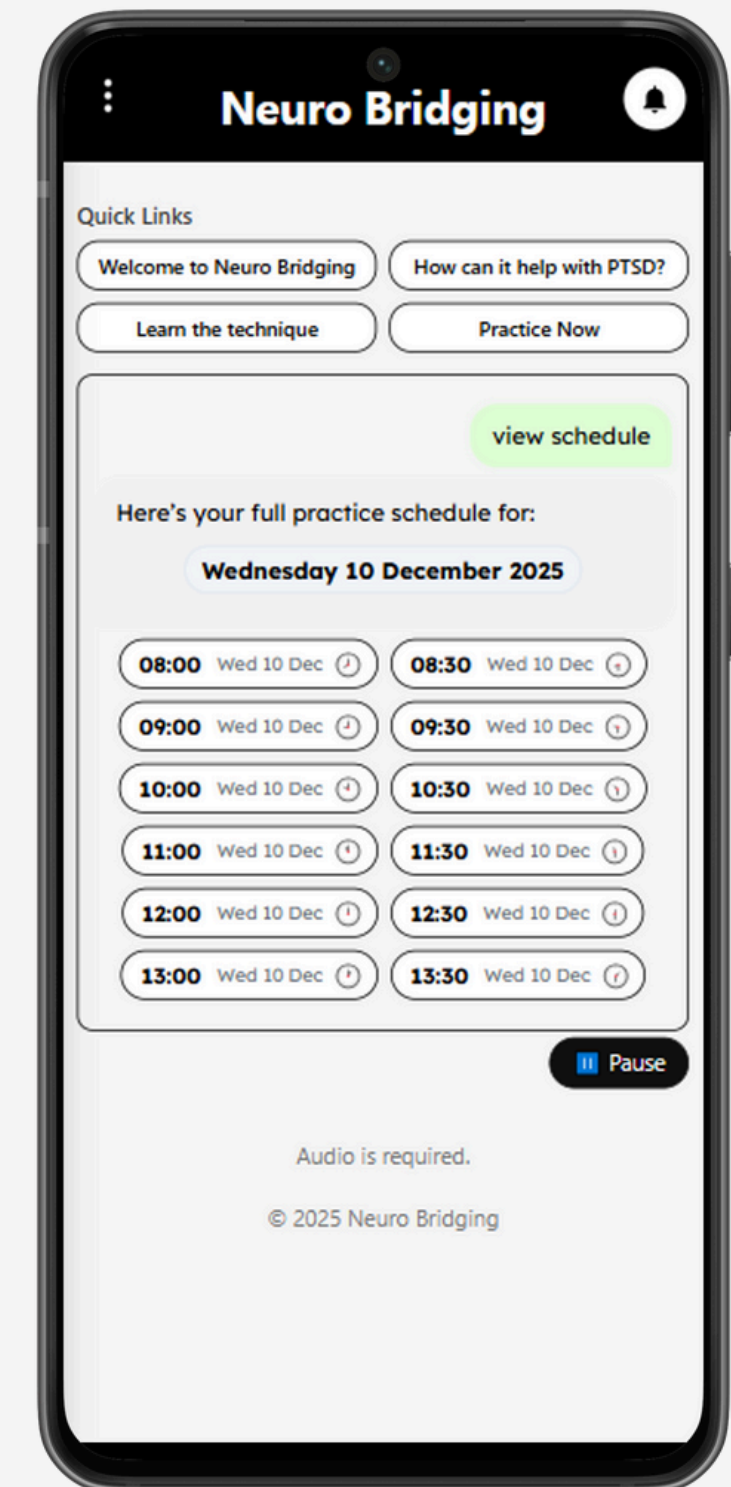
Continue

Not a diagnosis. If you're in immediate danger or feel at risk of harming yourself, call your local emergency number.

NEURO-BRIDGING SCHEDULER

Solving a common problem in therapy: homework non-compliance

- Sets up a Neuro-Bridging practice schedule across a 6-hour practice window
- Sets timings for each practice session
- Repeated practices are notified via email
- Individual practice timer
- Audio guidance for each practice



Why Timely Practice Is Non-Negotiable

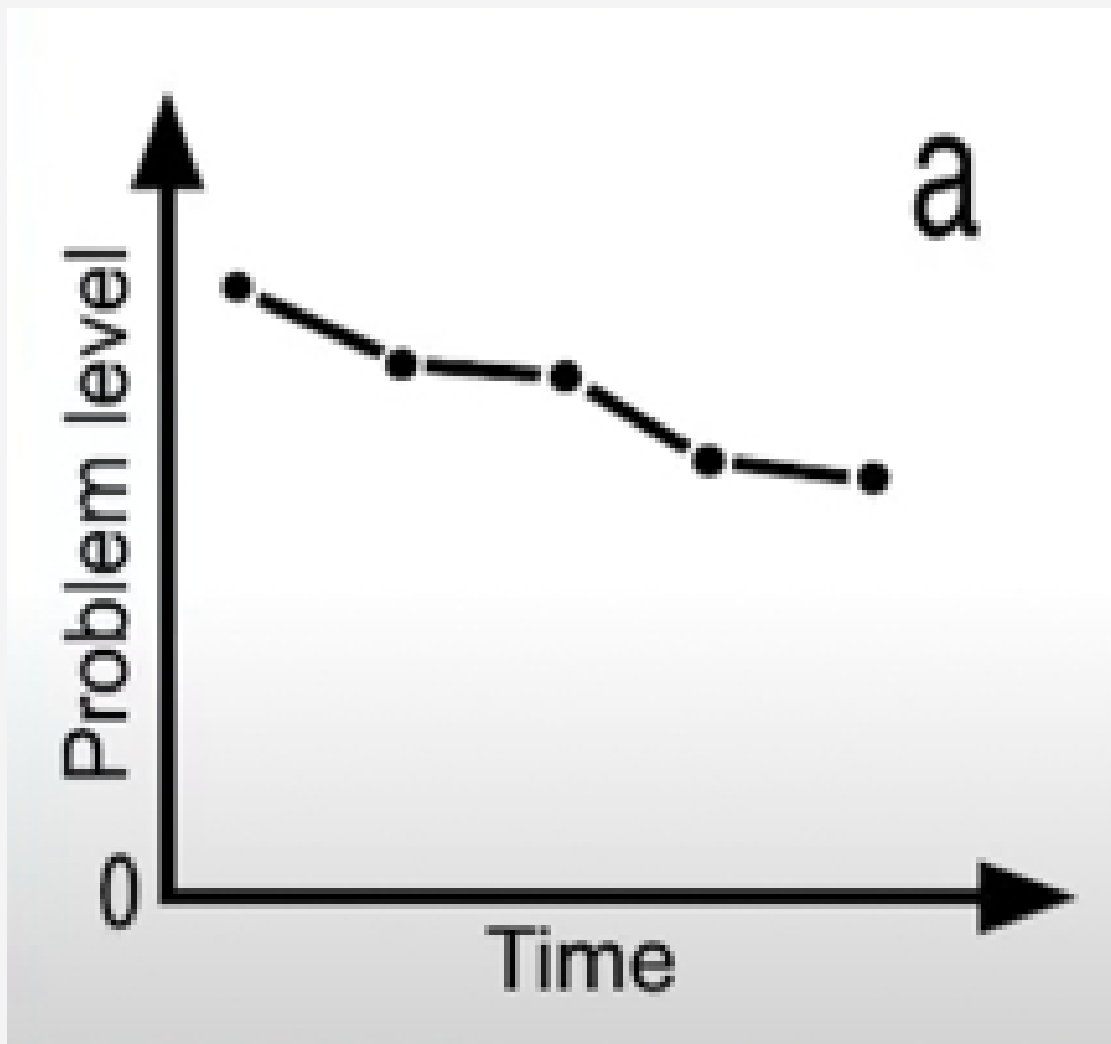
Neuro-Bridging Must Be Practised Within A 6-Hour Window

- Memory reconsolidation has a time limit
- Once a memory is reactivated, it becomes ‘editable’ for approximately 6 hours. This is the length of time needed for protein synthesis. After that, it ‘locks in’ again
- Updating emotional memories requires new proteins to be formed in the brain. This takes time and must occur within this critical window
- Emotional learning can only be rewritten during this window
- To dissolve flashbacks, nightmares and other symptoms the brain needs a new experience that directly contradicts the old learning and it must happen and be repeated within 6 hours of activation
- Miss the protein synthesis window, miss the transformation
- If users delay practice or don’t complete the sequence in time, the opportunity to rewire that memory passes and the symptom stays
- Our system guides users to activate, engage, and resolve emotional memories within the time window for memory reconsolidation
- Our system creates structure and timing

Without timely practice, even the best insights don’t lead to change. Our system ensures timing, repetition, and precision, so the brain can implement its own rules of transformational change.

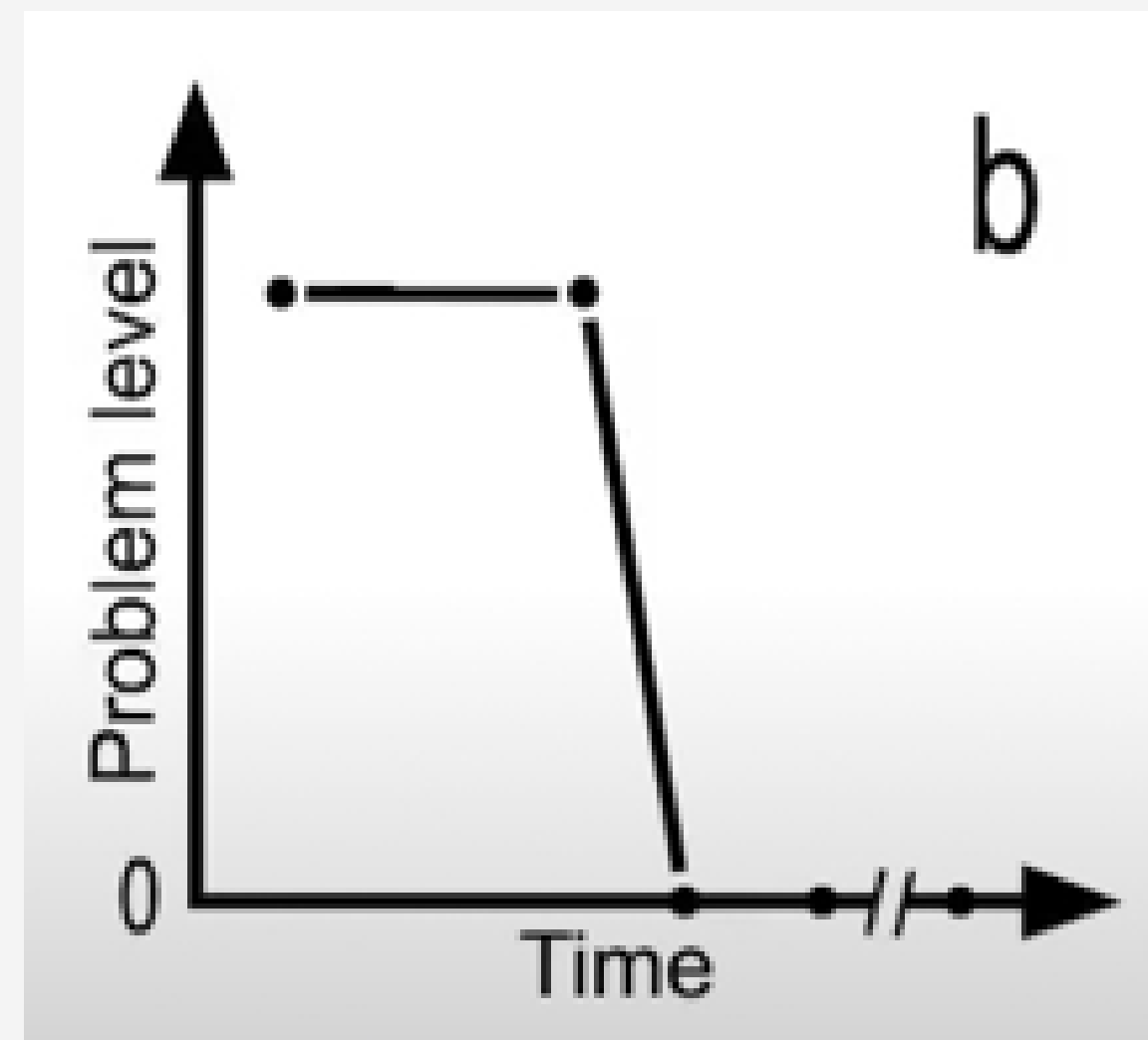
Key differentiator: Neuro-Bridging uses memory reconsolidation

Talking Therapies



Incremental change

Neuro-Bridging



Transformational change

MEET THE TEAM



Founder

John Joseph O'Brien

C.B.T. trained, John has thirty years of experience in clinical practice, specialising in addiction, addictive behaviours, stress and anxiety disorders. He has worked as both a lead therapist and manager at **Gladstone's rehab** facilities in both Bristol and Stroud.

John is currently in private practice where he sees individual clients. Neuro-Bridging is an integral part of John's clinical work.



Founder

Manuel Toren

Integrative Psychotherapist & Cognitive-Behavioural Therapist, B.Sc., M.A., PgDip., BABCP accred.

Manuel has done ground-breaking research in the area of functional disorders & mental health. A graduate of University College London (UCL) who trained in the **NHS** and has been working clinically for over 15 years, he is now lead clinician at the **Nightingale Hospital** OCD Unit.



Founder

Paul Newberry

With 25 years in software development, Paul co-created Neuro-Bridging after co-founding a fintech startup in the commodity space.

Paul spent 15 years at **Betfair** working across many parts of the organisation including the core trading exchange and martech



Legal

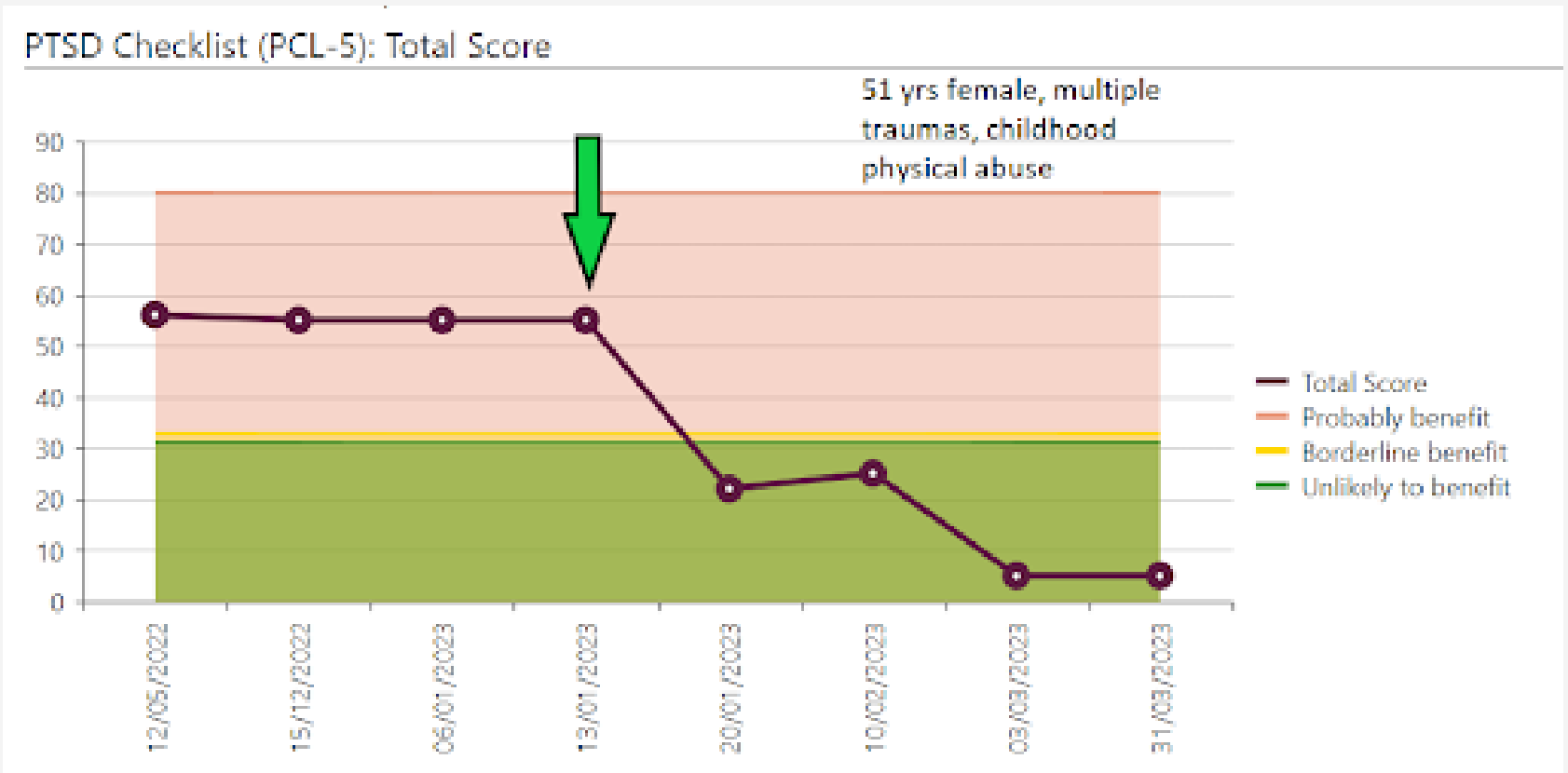
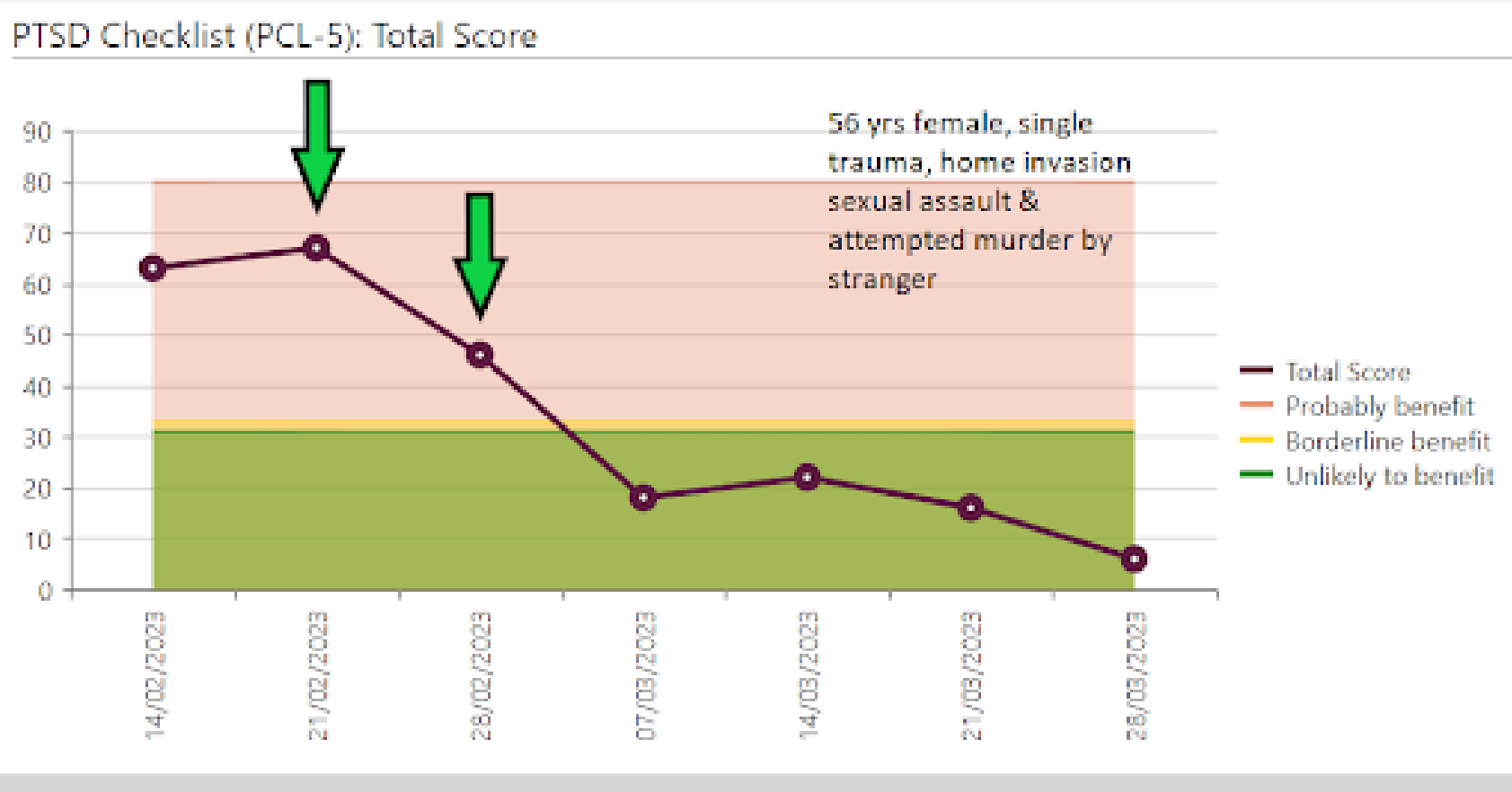
Martyn Bailey

Martyn is a partner at **Lee & Thompson** in the firm's Dispute Resolution group, specialising in entertainment, IP and commercial litigation.

Martyn has more than 25 years' experience in successfully resolving disputes for clients in a diverse range of industries

TRACTION

- The technique is currently being used in clinical practice with ground-breaking results for the treatment of PTSD.
- Our PTSD data set demonstrates a high success rate.



Examples of patient PCL-5 scores (arrow ₁₂ indicates use of Neuro-Bridging)

TRACTION - TESTIMONIAL

"Nothing changed in my living situation during those first few weeks, but from day one of trying Neuro-Bridging, I felt a shift. I referred to it as MAGIC. I stuck to the timings that were set out to me, and repeated the exercise religiously, and on day one, that was the first time I didn't have a nightmare or intrusive thought in months. I owe so much of my health now to those imagery exercises. I think it was a fantastic tool to confront my fears **without ever really discussing any details** of the sexual abuse."

A team forged from real-world
experience. Deeply understanding
the problem and uniquely
equipped to solve it.

THE MARKET-PEOPLE WE CAN HELP

\$38.42bn

\$1.29bn

\$167M

TAM

Revenue in the Mental Health market is projected to reach US\$38.42bn in 2024

SAM

- THE UK MARKET
- Revenue in the Mental Health market is projected to reach US\$1.29bn in 2024
- Revenue is expected to show an annual growth rate (CAGR 2024-2028) of 1.33%, resulting in a market volume of US\$1.36bn by 2028

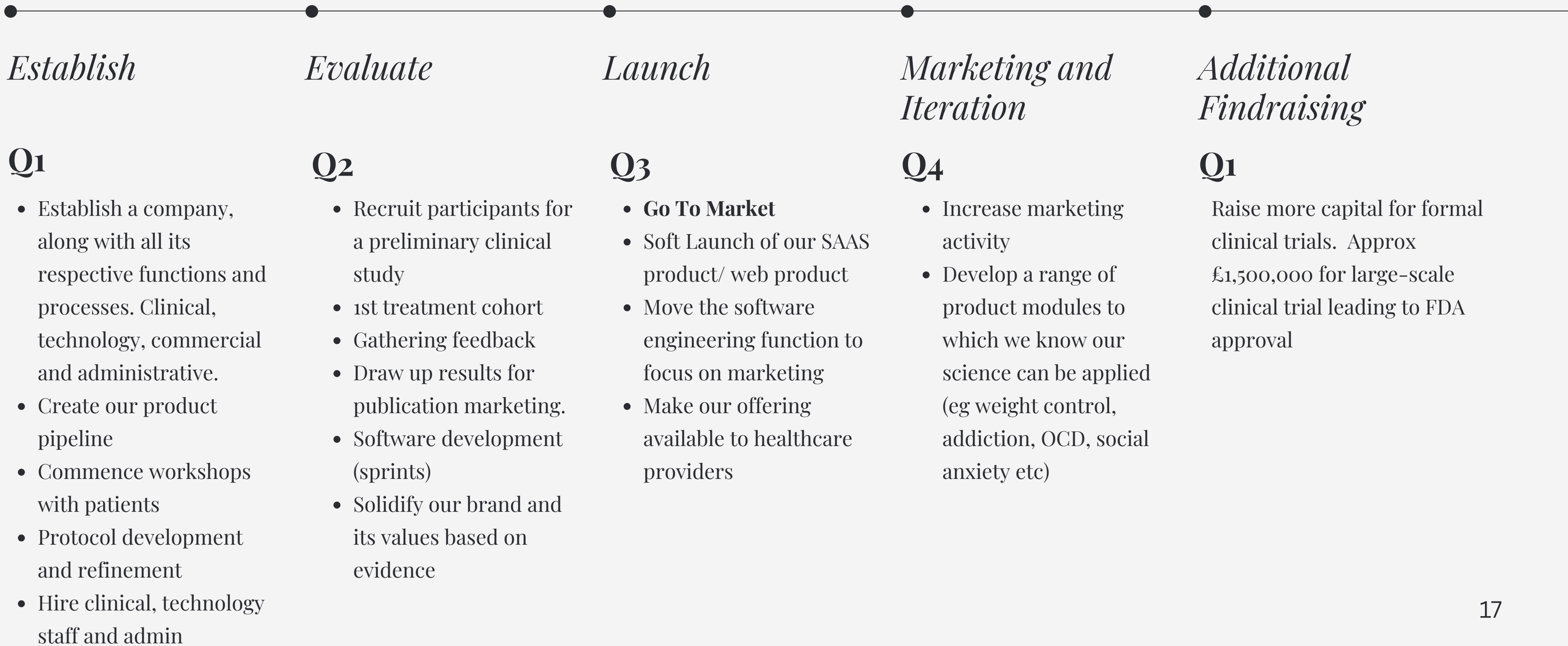
SOM initial target market

Revenue in the Mental Health market for the London area.

Potential Deployment Options

Deployment Option	Security & Privacy Notes	Speed of Deployment
Web App (Browser)	Centralized updates; HTTPS + WAF; minimal local data. SSO/OIDC easy. Track PII in logs & analytics.	Fast — minutes to days.
Mobile App (iOS/Android)	Device data at rest; MDM/biometric; secure storage (Keychain/Keystore).	Fast — minutes to days.
SaaS (Multi-Tenant Cloud)	Shared infra; strong tenant isolation; data residency choices; standard certifications (SOC 2/ISO27001).	Fast — hours to days (vendor signup).
Single-Tenant Cloud (VPC)	Data isolation; custom network controls; private links; higher cost/ops burden.	Moderate — days to weeks (provisioning).
On-Premises (Self-Hosted)	Full control; strict data locality; patching/DR on you; internal audit controls.	Slow — weeks to months (hardware, setup).
Offline / Air-Gapped	Max privacy; no external network; signed updates via removable media; strict supply-chain checks.	Fast — minutes to days.

ROAD MAP - GTM



THE FUTURE

OUR PRODUCT HAS THE POTENTIAL TO TREAT A WIDE RANGE OF MENTAL HEALTH PROBLEMS INCLUDING:

- ADDICTION
- STRESS
- SOCIAL ANXIETY
- OCD
- ANGER
- EMOTIONAL EATING

... *and where memory is the root cause*

FUNDING REQUIREMENTS

£ 2 MILLION / 18 MONTH RUNWAY

FUNDING ALLOCATION

- Ad hoc clinical space, treatment rooms
- Private Office space
- Wages
- Administration costs
- Preliminary clinical trial with curebase
- Software engineering
- Infrastructure (hosting, hardware, software)
- Technology wages / outsourcing costs

ROUND OBJECTIVES

- 4.5k clients treated
- A stable scalable organisation
- Published clinical results
- A final scalable software solution, with multiple product offerings. Mobile, web, SAAS, standalone, etc

**A solution that does
not rely on a
therapist**

**A PTSD solution that
can be delivered
remotely**



Created by Mark Jayvee Pabilonia
from Noun Project

OUR VISION

To transform the world of mental health treatment using our simple effective technique, drastically reducing treatment time and cost.

Using a flexible deployment model designed to reach anyone in need, anywhere in the world quickly and with minimal setup.

S U M M A R Y

- A groundbreaking therapeutic technique, which has already proven itself with results
- MVP and plan to develop into a high-quality go-to-market product
- A strong team of experts is already in place
- Multiple avenues to drive strong long-term growth
- The time is right to disrupt the global market with a unique transformative product

DATA SOURCES

https://www.england.nhs.uk/aac/wp-content/uploads/sites/50/2022/09/B1482_research-demand-signalling-national-mental-health-programme_september-2022.pdf

[https://www.thelancet.com/journals/langlo/article/PIIS2214-109X\(20\)30432-0/fulltext](https://www.thelancet.com/journals/langlo/article/PIIS2214-109X(20)30432-0/fulltext)th Programme

<https://pubmed.ncbi.nlm.nih.gov/35485933/>

<https://www.statista.com/outlook/hmo/mental-health/worldwide>

[https://www.statista.com/outlook/hmo/mental-health/united-kingdom#:~:text=Revenue%20in%20the%20Mental%20Health,US%2411%2C730.00m%20in%202024\).](https://www.statista.com/outlook/hmo/mental-health/united-kingdom#:~:text=Revenue%20in%20the%20Mental%20Health,US%2411%2C730.00m%20in%202024).)

<https://www.sciencedirect.com/science/article/abs/pii/S0005796706000477>

C O N T A C T

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